



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صفاء خميس عبدالله على

العنوان : 0

الحي / المدينة : ابوالنمرس

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 30-09-2026

رقم التليفون : 01143753055

رقم البطاقة : 27809022101329

العمر (تاريخ الميلاد) : 02-09-1978 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: صفاء خميس عبدالله علي National ID: 27809022101329	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Name: صفاء خميس عبد الله علي		GUSTAVE ROUSSY	
MR. : 2609			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها			
The patient was diagnosed with metastatic breast cancer (MBC) to bone, with estrogen receptor (ER) 8/8, progesterone receptor (PR) 8/8, and HER2 0. Initial management included an aromatase inhibitor (AI) combined with zoladex, resulting in complete resolution of disease. The patient subsequently underwent a left total mastectomy with radical modified (LT MRM) on 20-10-2025. The record notes "ypt2 Ypn2," and indicates that the patient has not received chemotherapy (CTh) or postoperative radiation therapy (PORT). The patient presented today for administration of zoladex and continuation of femara for a planned three-month course. The patient reports bony aches, likely arthritic in nature, and the plan is to monitor these symptoms with computed tomography (CT) scans after three months. The patient attended today for administration of femara and zoladex. The patient was scheduled for a CT of the chest, abdomen, and pelvis (CT CAP) in preparation for zoladex administration on 13/10. A dual-energy X-ray absorptiometry (DXA) scan performed in July 2026 demonstrated normal bone density, with the lowest T-score recorded at -0.4.			
تحريراً في ,			
		التاريخ : 19-09-2026	الوقت :
		Reham Ahmed Fathy Ibrahim	
		الطبيب المعالج :	
يعتمد ,			



Patient Name : الأستاذة / صفاء خميس عبد الله	Request No. : 13002399
Patient No. : 27809022101329	Request Date : 17/08/2024
Path No. : 2980/24	Verification Date : 26/08/2024
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 45 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical data

Left breast palpable mass

Nature of specimen

Tru cut biopsy, collected at 17/8/2024, 10:30 am

Gross examination

Multiple(3) grayish white tissue cores up to 2 cm, totally submitted.

Microscopic examination

- * Histological type: Invasive lobular carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angioinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No

Diagnosis

Left breast mass, Tru cut biopsy:

Invasive lobular carcinoma, Grade II.

For further IHC (E-cadherin ,ER, PR, HER2, Ki67)

Signature

Dr. Salma Tarek

Dr.Heba Hassan

Verified by :

Dr.Heba Hasan





Patient Name	: الأستاذة / صفاء خميس عبد الله	Request No.	: 13002399
Patient No.	: 27809022101329	Request Date	: 17/08/2024
Path No.	: 2980/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - ميلاده
Age	: 45 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast palpable mass.

Nature of Specimen: Tru-cut biopsy, collected at 17/8/2024, 10:30 am.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:

Invasive lobular carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between

Verified by :

Dr. Sagda El-Kady





Patient Name	الأستاذة / صفاء خميس عبد الله	Request No.	13002399
Patient No.	27809022101329	Request Date	17/08/2024
Path No.	2980/24	Verification Date	02/09/2024
Sex	Female	Financial Category	اورام دار السلام - ميانره
Age	45 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Gamal Abdelatty.
Dr. Menna Nabil.

Verified by :
Dr. Sagda El-Kady



Patient Name	: الأستاذ / صفاء خميس عبد الله	Request No.	: 13007971
Patient No.	: 27809022101329	Request Date	: 21/10/2025
Path No.	: 3223/25	Verification Date	: 09/11/2025
Sex	: Female	Financial Category	: جوماتاف روسي - مبادر
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post-neoadjuvant hormonal therapy.
Nature of specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 24x20x5 cm, with covering skin ellipse measured 20x18 cm. Nipple & areola looked unremarkable grossly.
- * Serialing showed a tumor bed site measured 5x5x1.5 cm, located 2.5 cm away from the deep margin.
- * Axilla: Axillary fat measured 7x7x1 cm, entangling 12 lymph nodes, the largest was 1.5x1.5x1 cm.
- 12 Blocks:
- 1:6- T.B.S., 7- N&A., 8- Deep margin., 9:12- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic examination:

- * Histological type: Residual viable invasive classic lobular carcinoma (Actual tumour size 5x5 cm) (Cellularity 15%).
- * Grade 1 (differentiation 3, nuclear pleomorphism 1, mitosis 1).
- * Ductal carcinoma in situ: Absent.
- * Lobular carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angio-Invasion: Present.
- * Additional findings: Intraductal papilloma with microcalcification.
- * Nipple and areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Viable metastatic deposits involving six out of twelve lymph nodes, L.Ns (6/12) (The largest nodal mets measuring 1.5 cm, with focal extra-nodal extension).
- * Histologic response to therapy: Pathologic partial response to therapy.

Verified by :



Patient Name :	الأمثلة / صلاء خريس عبد الله	Request No. :	13007971
Patient No. :	27809022101329	Request Date :	21/10/2025
Path No. :	3223/25	Verification Date :	09/11/2025
Sex :	Female	Financial Category :	جومات روسي - مبدرة
Age :	47 Y	Department :	Out Patient
Referring Doc :		Registered by :	marwa mohamed youss

Pathology Report

Diagnosis:

Left breast cancer (Post-Neoadjuvant hormonal therapy), Left modified radical mastectomy:
Residual viable invasive classic lobular carcinoma, Grade 1 (Cellularity 15%) (Actual tumor size 6x5 cm).

- * Ductal carcinoma in situ: Absent.
- * Lobular carcinoma in situ: Absent.
- * Nipple and areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Viable metastatic deposits involving six out of twelve lymph nodes, L.Ns (6/12) (The largest nodal mets measuring 1.5 cm, with focal extra-nodal extension).
- * Stage: ypT2, ypN2a.

Dr. Walaa Ibrahim Ghonem.
Dr. Duaa Mohamed.

Signature

Verified by :



Patient Name: Safaa Khamis Abd Aliah Ali Elewa	Date : 07/05/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 48 Year(s)
Medical ID: 2893/2024	Pat. ID: 77440

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows LIQ parenchymal distortion
- Complementary ultrasound revealed:
- Left breast:
- Left 3-4 o'clock zone B (4cm from nipple) small irregular hypoechoic lesion measuring about 1.5x0.8cm as compared to 3.6x2.5 cm previously
- The left UOQ alerted parenchyma is not currently appreciated
- Still noted left axillary level I multiple pathological LNs largest measured about 1x1.8 cm with focal cortical thickening reaching 8mm
- Free left infra and supra-clavicular regions.
- Right breast:
- No right breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific right axillary LNs

Opinion:

- Partial radiological response of the left breast 3-4 o'clock malignant mass (BIRADS 6)
- Still noted left axillary level I multiple pathological LNs.
- Benign findings right breast (BIRADS 2)

Dr. basma mohamed